

بسم الله الرحمن الرحيم

عشم باكر

Bawasig pediatric exam

1. While examining a five year old boy, collapsing peripheral pulses were detected.

What is the most likely diagnosis?

- A. Mitral regurgitation.i
 - B. Aortic incompetence.**
 - C. Huge goiter with euthyroid status.
 - D. Pulmonary hypertension.
 - E. Large VSD.
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2. An eight month- old child presented with shortness of breath and inability to feed. Examination revealed marked pallor and a respiratory rate (RR) of 55/ min, a pulse rate of 120/min. Liver was palpable 6 cm below the costal margin.

Which of the following indicates a diagnosis of heart failure?

- A. Loud second heart sound.
- B. Small volume peripheral pulses.
- C. Excessive sweating at rest.**
- D. Short apical diastolic murmur.
- E. Left parasternal heave .

3. A seven year-old boy was referred with facial puffiness and turbid urine for one week. Examination revealed a normal blood pressure. Urinalysis showed +++ of albumin, ++ pus cells, 1-2 RBCs with granular casts. His renal function test was normal.

What is the most likely diagnosis?

- A. **Minimal change nephrotic syndrome.**
 - B. Acute Glomerulonephritis.
 - C. IgA nephropathy .
 - D. Urinary tract infection (UTI).
 - E. Hemolytic uremic syndrome.
4. Which one of the following murmur characteristics supports the diagnosis of an innocent murmur?
- A. Short ejection systolic murmur.
 - B. Association with a palpable thrill.
 - C. **Variability with position and posture.**
 - D. Radiation to the neck.
 - E. Maximum intensity at the left lower sternal border.
5. A four- year old boy has waddling gait, lumbar lordosis, and a positive Gower s sign.

What is the initial investigation to reach a diagnosis?

- A. Serum electrolytes.
- B. Thyroid function test.
- C. Muscle biopsy.

D. Cardiac echocardiogram.

E. Creatine Phospho Kinase (CPK) level

6. A three months old infant was brought to ER having episodes of body stiffening, associated with head and upper limb flexion on awakening, not associated with loss of consciousness but are followed by crying, occurring frequently for the last one month. Delayed social smile was noted.

What is the most likely diagnosis?

A. Infantile spasms.

B. Intussusception.

C. Infantile colic.

D. Renal colic.

E. Meningitis.

7. A five months old male was brought by his mother to the outpatient department because he can not support his head yet. During pregnancy reduced fetal movements were noted. O/E head circumference was normal but he was noted to be hypotonic with tongue fasciculations.

What is the most important diagnosis to consider?

A. Cerebral palsy.

B. Encephalitis.

C. Kernicterus.

D. Congenital TORCH infection.

E. Spinal muscular atrophy (SMA).

8. A four Kg neonate was born to a diabetic mother and was admitted to neonatal intensive care unit for observation.

What is the most urgent investigation required ?

- A. Serum Calcium.
- B. Cardiac echocardiogram.
- C. Blood glucose.**
- D. Serum bilirubin.
- E. Blood culture.

9. A six week-old infant was noted to have ambiguous genitalia. Serum electrolytes revealed hyponatremia and hyperkalemia.

Which one of the following is the most **specific** investigation to **confirm** a diagnosis?

- A. Karyotyping.
- B. 21- hydroxylase level.**
- C. Pelvic ultrasound.
- D. Serum urea and creatinine.
- E. Testosterone level.

10. An 11 year old male, known to be diabetic for 5 years was brought to the ER with diabetic ketoacidosis in coma. His dehydration was corrected using intravenous fluid therapy.

What is the next step in his management?

- A. Intravenous potassium chloride therapy.
- B. Intravenous dextrose 5% with half saline.**
- C. Intravenous sodium bicarbonate.
- D. Insulin infusion.
- E. Intravenous ceftriaxone.

11. A four months old infant was noted by his parents to have a burnt sugar smell of urine.

What is the most likely diagnosis?

- A. Homocystinuria.
- B. Fructosuria.**
- C. Glycogen storage disease.
- D. Maple syrup urine disease.
- E. Organic aciduria.

12. A young infant was rushed to the emergency room. He was noted to be cyanosed, gasping and has a heart rate of 40 beats per minute.

Which of the following is the initial intervention?

- A. Intraosseous cannulation.
- B. Synchronized cardioversion.
- C. Chest compressions alone.
- D. 100% oxygen by face mask.
- E. Bag-mask ventilation and chest compressions.**

13. A concerned mother who lives in a flat, brought her two year old child because he has not walked independently yet. He is active and bright, and sat at 6 months. On examination he has frontal bossing; he has mild hypotonia, and only 4 incisors.

What is the most likely diagnosis?

- A. Hydrocephalus.
- B. Cerebral palsy.
- C. Rickets.**
- D. Osteogenesis imperfecta.
- E. Hypothyroidism

14. Which of the following diseases is inherited as autosomal recessive?

- A. Marfan syndrome
- B. Sickle cell anemia.**
- C. Von willebrand disease.
- D. Glucose-6-phosphate-dehydrogenase deficiency(G6PD)
- E. Hemophilia B.

15. A nine months old infant was evaluated for bleeding after circumcision. Hemophilia A was confirmed.

Which family members do you expect to be affected?

- A. Maternal uncle.**
- B. Father.
- C. Sister.

- D. Paternal aunts.
- E. Maternal aunts.
16. Which one of the following is true regarding bronchiolitis?
- A. It is common in the summer months.
- B. It is commonly complicated by respiratory failure.
- C. It is caused by respiratory syncytial virus (RSV).**
- D. Intravenous antibiotics are indicated.
- E. All children require inpatient hospital management.
17. A 10 years old child who is known to have Asthma, presented with shortness of breath. On admission to ER, his RR was 60 breaths / min. he had pulsus paradoxus and a wheezy chest.
- What is the first step in his management?
- A. Oral prednisolone.
- B. 100% Oxygen.**
- C. Intravenous fluids.
- D. Intravenous Aminophylline.
- E. Adrenaline.
18. A ten year-old female presented with severe headache, blurred vision and peri-orbital swelling for four days. Urinalysis showed uncountable red blood cells, and granular casts.
- I Which one of the following complications has she developed?

- A. Chronic renal failure.
 - B. Meningitis.
 - C. Brain abscess.
 - D. Idiopathic intracranial hypertension.
 - E. Hypertensive encephalopathy.**
19. A three year-old child with nephrotic syndrome, developed severe abdominal pain and tenderness.
- Which one of the following complications is the most likely diagnosis?
- A. Intussusception.
 - B. Acute cholecystitis.
 - C. Appendicitis
 - D. Bacterial peritonitis.**
 - E. Acute renal failure.
20. An eight month-old infant presented with fever, and generalized tonic clonic convulsions. On examination his temperature was 40°C, Glasgow coma score was 15 and he had a bulging anterior fontanel.
- What is the most likely diagnosis?
- A. Febrile convulsion.
 - B. Meningitis.**
 - C. Encephalitis.
 - D. Mastoiditis.

E. Septicemia.

21. A five days old neonate presented with fever and refusal of feeding. His mother had drainage of liquor 48hours prior to delivery.

What is the most likely causative organism for his condition?

A. Group B streptococci.

B. Neisseria meningitides.

C. Hemophilus influenza.

D. Staphylococcus aureus.

E. Candida albicans.

22. A term neonate developed jaundice on day one of life. Mother's blood group is O +ve, baby's blood group is B+ve, Hb 10 gm/dl, total serum bilirubin 15 mg/dl. Direct Coombs test was +ve.

What is the most likely cause of this baby's jaundice?

A. Rhesus incompatibility.

B. ABO incompatibility.

C. Breast milk jaundice .

D. Physiological jaundice.

E. Spherocytosis.

23. A Down syndrome baby developed bilious vomiting and abdominal distention on the second day of life. X- ray abdomen showed double bubble sign.

What is the most likely diagnosis?

- A. Hirshsprung disease.
 - B. Gut malrotation.
 - C. Pyloric stenosis.
 - D. Jejunal atresia.
 - E. Duodenal atresia.**
24. A three year-old child who is a known sickler presented with sudden pallor and abdominal pain. Abdominal examination revealed a huge spleen. CBC revealed pancytopenia.
- What is the most likely diagnosis?
- A. Splenic Sequestration crisis.**
 - B. Hyperactive malarial syndrome (HMS).
 - C. Acute cholecystitis.
 - D. Aplastic crisis.
 - E. Infectious mononucleosis.
25. Which one of the following is a true combination of the pentavalent vaccine?
- A. Polio/diphtheria/measles/BCG/rotavirus.
 - B. DTP/Hepatitis B virus/BCG.
 - C. Hepatitis B/Hepatitis C/Hepatitis A/typhoid/cholera.
 - D. Hemophilus influenza/ rotavirus/DTP.
 - E. DTP/Hepatitis B/hemophilus influenza.**

26. A five year-old boy presented with ascending acute flaccid paralysis two weeks after an upper respiratory tract infection.

Which one of the following is the first line of management?

- A. Intravenous steroids.
- B. Intravenous immunoglobulin.
- C. Intravenous broad spectrum antibiotics.**
- D. Physiotherapy only.
- E. Plasmapheresis.

27. A two years old child was brought by his mother for developmental evaluation.

Which one of the following skills is expected for his age?

- A. Drawing a circle.
- B. Undoing shoe laces.
- C. Buttoning his shirt.
- D. Making a two word sentence.**
- E. Riding a tricycle.

28. A child can draw a circle, but not a square. He can name 3 body parts.

What is the likely age in years?

- A. 2
- B. 3 in**
- C. 4

D. 5

E. 6

29. A one- year old child presented with a flu like illness; on day two he developed a barking cough and inspiratory stridor. O/E, he looks well, has a Temp of 38.5°C, and he has no drooling of saliva.

Which one of the following is the most likely diagnosis?

A. Acute epiglottitis.

B. Foreign body inhalation.

C. Acute laryngo-tracheo-bronchitis.

D. Laryngeal diphtheria .

E. Acute bronchiolitis.

30. A six week-old infant presented with a four- week history of jaundice. Total bilirubin was 18mg/dl, with indirect bilirubin of 15mg/dl. His blood group is O positive.

Which one of the following conditions is the most likely diagnosis?

A. Biliary atresia.

B. ABO incompatibility.

C. Breast milk jaundice.

D. Spherocytosis.

E. Rhesus incompatibility.

31. A seven years old girl was noted to have stunted growth, abdominal distention and a history of diarrhea. Peripheral blood picture revealed microcytic hypochromic anemia.

Which one of the following tests is needed to reach a diagnosis?

- A. Barium meal and follow through.
 - B. Left wrist X-ray.
 - C. Tissue transglutaminase antibodies.**
 - D. Fecal fat test.
 - E. Ultrasound abdomen.
32. What is the most common cause of acute hepatitis in an 8 years old child?
- A. Hepatitis A.**
 - B. Hepatitis B.
 - C. Hepatitis C.
 - D. Yellow fever.
 - E. Epstein-Barr virus.
33. A three year-old boy presented to the ER after ingestion of kerosene. He had stable vital signs including normal oxygen saturations.
- What is the next plan of action regarding his management?
- A) Gastric lavage.
 - B) Intravenous steroids and antibiotics.
 - C) Ipecac syrup via nasogastric tube.
 - D) Admission and observation.**
 - E) Activated charcoal.

34. A 14 years old female presented with small joints pain and swelling, a malar rash, weight loss and alopecia.

Which one of the following tests confirms the diagnosis?

- A. Antineutrophil cytoplasmic antibody(ANCA).
 - B. Rheumatoid factor.
 - C. **Anti-double stranded DNA antibodies (anti-dsDNA).**
 - D. Anti-smooth muscle antibodies.
 - E. ESR
35. Which one of the following is a characteristic feature of juvenile idiopathic rheumatoid arthritis (JIRA)?
- A. **Salmon colored rash that flares with fever.**
 - B. Disfiguring arthritis.
 - C. Photosensitive rash.
 - D. Heliotrope rash.
 - E. Raynaud's phenomenon.

36. A child can walk with support, has stranger anxiety and says mama and dada.

What is the expected age of this child in months?

- A. 9
- B. **10**
- C. 12
- D. 14
- E. 15

37. Regarding supplementary feeding practice for six months old infants

Which one of following statements is the best advice?

- A. Cereal foods are better than starchy foods to start with.
- B. Eggs can be introduced at any age.
- C. Mashed vegetables can be started early.**
- D. Honey can be introduced at 7 months of age.
- E. Feeding the child with the rest of the family is recommended.

38. A two years old healthy child presented with watery diarrhea for 4 days. He was assessed using WHO criteria and was found to be irritable , with sunken eyes and drinking eagerly when offered fluids.

What is the appropriate management of his condition?

- A. 75ml/kg of ORS over 4 hours .**
- B. 100ml/kg of Ringer Lactate over 3 hours.
- C. 75ml/kg ReSoMal over 4 hours.
- D. 20ml/kg intravenous normal saline.
- E. 100ml/kg dextrose saline over 3 hours.

39. An 18 month-old child presented with failure to gain weight. On examination his weight was 6kg (less than 60% of his 50th centile), with lower limb pitting oedema.

How do you classify the condition of this child according to the Welcome classification?

- A. Under weight.

- B. Normal weight
 - C. Kwashiorkor.
 - D. Marsmic kwashiorkor.**
 - E. Marasmus .
40. A 3 years old child presented with fever, weight loss and epistaxis . On examination he is pale, has generalized lymphadenopathy and hepatosplenomegaly. Investigations revealed a WBC of $65,000/\text{mm}^3$, Hb 4.5g/dl , platelets $60,000/\text{mm}^3$.
- What is the most likely diagnosis?
- A. Acute lymphoblastic leukemia.**
 - B. Chronic granulocytic leukemia(CGL)
 - C. Non Hodgkin lymphoma.
 - D. Visceral leishmaniasis.
 - E. Infectious mononucleosis.
41. Which one of the following signs is a characteristic prodromal sign of measles infection?
- A. A maculopapular rash on the face and trunk.
 - B. Crops of pustular lesions on the face and scalp.
 - C. Koplik spots on the mouth.**
 - D. Posterior cervical lymphadenopathy.
 - E. Red, cracked lips.

42. A seven years old child from Gadarif State presented with fever and epistaxis for 2 months. Physical examination showed an ill and pale child . Liver was 2 cm below the costal margin and the spleen was 14 cm below the costal margin. Hemoglobin level was 5 gm/dl.

Which one of following is the most likely diagnosis?

- A. Hodgkin s Lymphoma .
- B. Brucellosis.
- C. Portal hypertension.
- D. Disseminated tuberculosis.

E. Visceral leishmaniasis.

43. A child presented to the emergency room with fever and vomiting. His blood film for malaria was positive.

Which one of the following investigations indicates severe malaria?

- A. C- reactive protein (CRP) more than 50.
- B. ESR more than 100 mm/hr.
- C. Parasite count less than 5%

D. Hemoglobin of 4 gm/dl.

- E. Reticulocyte count of 3%.

44. Which one of the following complications is expected in a child with chronic otitis media with effusion?

- A. Meningitis.
- B. Mastoiditis.

- C. Permanent nerve deafness.
- D. Brain abscess.
- E. **Impaired speech and language development.**

45. Which one of the following is a priority in emergency triage?

- A. **Obstructed breathing.**
- B. Jaundice
- C. Tachycardia
- D. Peripheral cyanosis.
- E. Fever with rigors.

46. Which one of the following micronutrients should not be given in the initial management of a malnourished child?

- A. Folic acid.
- B. **Vitamin D.**
- C. Iron.
- D. Vitamin A.
- E. Zinc sulphate.

47. A five years old child presented with nocturnal anal itching. His brother had the same symptom.

What is the likely causative organism for this condition?

- A. Ascaris Lumbricoides.
- B. Entrobilus vermicularis.
- C. Candida albicans.
- D. Treponema pallidum.
- E. **Tinea saginata.**

48. Which one of the following combinations of drugs and their commonly observed side effects is true?
- A. Prednisolone and weight loss.
 - B. Carbamazepine and hepatotoxicity.
 - C. **Co-trimoxazole and skin rash.**
 - D. Streptomycin and optic neuritis.
 - E. Omega 3 and nephrotoxicity.
49. A 3 year old child presented with speech delay, hyperactivity, stereotyped body movements and loss of eye contact. His clinical examination was normal with no focal deficits. MRI (Magnetic Resonance Imaging) of the brain was normal.
- Which one of the following is the most likely diagnosis?
- A. **Autistic spectrum disorder (ASD).**
 - B. Familial speech delay.
 - C. Hyperactivity disorder.
 - D. Anxiety disorder.
 - E. Temper tantrums.
50. A mother of a child with cerebral palsy is asking you about stem cell transplant to try to achieve a cure for her disabled child. You have never heard about that form of treatment.
- What will be your response to her question?
- A. This form of treatment is not available in Sudan; so you do not need to ask about it.
 - B. **I have not heard about this form of treatment before; I need to go and read about it and get back to you.**
 - C. You disregard her question.
 - D. No treatment will ever benefit a child with cerebral palsy.
 - E. This is an excellent modality of treatment; please go ahead.

بالتوفيق

